

AHN Forbes Hospital

2570 Haymaker Rd, Monroeville, PA 15146

DISCHARGE SUMMARY Document #: DIS-007

Patient & Admission

Name	Elizabeth Harvey
Date of Birth	1975-01-25
Phone	724-555-9763
Address	7738 Leon Underpass Apt. 148 Pittsburgh, PA, 15222
MRN	MRN-433200
Admit Date	2026-05-06
Discharge Date	2026-05-12
Attending	Amy Campos, MD
Attending NPI	2154303913

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	JIM514846564

Primary Discharge Diagnosis

Acute STEMI, anterior wall

ICD-10: I21.4, I25.10

Hospital Course

The patient presented to the emergency department with acute anterior wall ST-elevation myocardial infarction. He reported sudden onset of severe substernal chest pain radiating to the left arm and jaw, accompanied by diaphoresis and dyspnea. Initial electrocardiography demonstrated ST elevation in the precordial leads V1 through V4 with reciprocal changes inferiorly. Troponin I was markedly elevated at 8.2 ng/mL. The patient was immediately taken to the cardiac catheterization laboratory where angiography revealed complete occlusion of the left anterior descending coronary artery with TIMI 0 flow. Primary percutaneous coronary intervention was performed with successful placement of a drug-eluting stent to the LAD, restoring TIMI 3 flow. Post-intervention angiography showed excellent stent apposition with no residual stenosis.

Transthoracic echocardiography performed on hospital day two revealed anterior wall akinesis with a moderately reduced left ventricular ejection fraction of 38 percent. The patient was initiated on guideline-directed medical therapy including dual antiplatelet agents, high-intensity statin, ACE inhibitor, and beta-blocker. His hospital course was complicated by one episode of nonsustained ventricular tachycardia on day three, which was self-limited and did not recur with optimization of electrolytes and beta-blockade. BNP level was 487 pg/mL, consistent with acute decompensation. Diuretics were administered judiciously given

the acute infarction, and the patient's volume status improved steadily. He remained hemodynamically stable throughout the remainder of his stay with no recurrent ischemic symptoms.

The patient was discharged on hospital day six in stable condition with close outpatient cardiology follow-up scheduled for one week. He was counseled extensively on medication adherence, cardiac rehabilitation enrollment, and lifestyle modifications including smoking cessation, dietary sodium restriction, and gradual return to activity. Repeat echocardiography is planned in three months to reassess ventricular function and guide further therapeutic decisions.

Procedures Performed

Coronary angiography	CPT 93458
PCI to RCA with DES	CPT 92928

Medication Changes

Atorvastatin 80 mg daily	STARTED
Clopidogrel 75 mg daily	STARTED
Ticagrelor 90 mg BID	STARTED
Aspirin 81 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	8 days
Urgency Tier	MEDIUM
Clinical Flags	post-acute-MI, EF<40%, post-PCI dual antiplatelet therapy

Dictated and electronically signed by:
Amy Campos, MD
NPI: 2154303913
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